Gemcitabine 800 mg/m² · 28-day schedule

NCCP 00749 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your practical treatment plan

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan.

STEP 1

Before the first visit

Blood tests, medicines review and the consent discussion come first. Review previous treatment, organ function and the treatment-day blood-test plan.

STEP 2

Plan venous access

A cannula, PICC or implanted port may be used. If a port is chosen, a trained team inserts it in a separate procedure with local anaesthetic and sometimes sedation; the team explains fasting, blood-thinner and wound instructions.

STEP 3

Infusion day

Staff check the line, give prescribed premedicines and treatment, and observe for reactions. Tell them immediately about burning, pain, rash or breathlessness.

STEP 4

Home and next review

This regimen does not routinely need a take-home infusion pump. Staff disconnect the infusion; a port is de-accessed when appropriate. Keep line-care and next-visit instructions.

Port insertion and removal

The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.

PICC care and urgent line symptoms

A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.

Visit time and transport

Allow time for blood review, preparation, treatment and observation. Your unit confirms the full visit duration. Arrange transport or an escort where advised.

Phase-specific visit pattern

Gemcitabine 800 mg/m² is given on Days 1, 8 and 15 of each 28-day cycle, for six to eight cycles unless progression or unacceptable toxicity changes the plan. No take-home infusion pump is part of this schedule. Port de-accessing after a visit is different from removal of the implanted port.

Sources: NCCP 00749 v3 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.